

العمر(تاريخ الميلاد) : 06-05-1969 المهنة : ربه منزل

التوقيع: (_____)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: عزه فاروق عبد الحفيظ علي National ID: 26905060102148	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

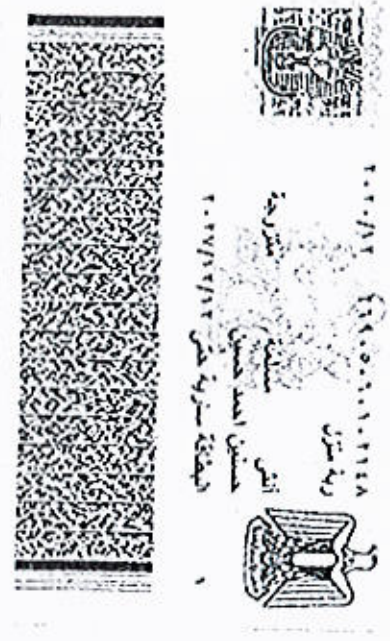
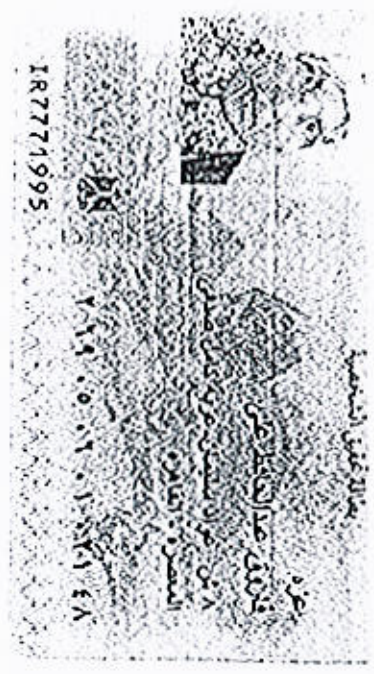
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
وزارة الداخلية
بطاقة الهوية الشخصية



عزده
فاروق عبد الحافظ على
٨ ش. عواد سليمان - عزبة كدلى صدف
المعصرة - القاهرة

١٩٦٩/٠١/٠١

٢٦٩٠٥٠٦٠١٠٢١٤٨

IR7771995

Name: عزة فاروق عبدالحافظ		
MR. : 1936		
Detailed Medical Report		
		السيد
تخية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient is a 56-year-old individual with a history of post-mastectomy right breast cancer. Imaging on 1/11/2022 revealed a right breast mass with ypT2N1 status and lymph node involvement (1/6). Pathology was invasive ductal carcinoma, ER 8/8, PR 5/8, Ki-67 26%. The patient received adjuvant chemotherapy with AC for 12 weeks followed by taxol. Hormonal therapy with Femara was initiated in July 2023. Imaging surveillance includes a scheduled MRI of the brain in August 2025 and a CT chest in August 2025. Full laboratory workup, including a lipid profile, is planned. Calcium supplementation with PAUS Calcium 1*1*90 and ONE ALPHA is prescribed. The most recent note indicates that a CT chest is pending, and the patient continues on Femara with calcium supplementation (PAUS Calcium 1*1*90, ONE ALPHA). No explicit cycle or week markers for any regimen are documented in the notes.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : ايه مدحت بدوي محمد	
يعتمد ,		

Patient Name : الأستاذة / عزه فاروق عبدالعظيم
Patient No. : 26905060102148
Path No. : 2542/22
Sex : Female
Age : 53 Y
Referring Doc. :

Request No. : 13009414
Request Date : 31/08/2022
Verification Date : 18/10/2022
Financial Category : أورام دار السلام - خلية الدولة
Department : Out Patient
IHC No. :
Registered by : Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical data: Right breast lesion.

Nature of Specimen: Tru cut biopsy.

Gross:

- Multiple grayish white tissue cores, longest measured up to 1 cm long. Totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic:

histopathological examination of sections prepared from the specimen received reveals cores of an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].

- IPT:
 - ER: Strong nuclear reaction in 100% of tumor cells (Allred score 8/8)
 - PR: Strong nuclear reaction in 20% of tumor cells (Allred score 5/8)
 - HER2: No membranous reaction in tumor cells (score 0).
 - Ki 67: 25% nuclear reaction of tumor cells.
- Positive control showed appropriate reactivity.

Diagnosis:

- Right breast, Tru cut biopsy;
- Invasive duct carcinoma, GII.
 - IPT:
 - ER: Positive (score 8/8).
 - PR: Positive (score 5/8).
 - HER2: Negative (score 0).

Verified by :

Dr. Marwa mahmoud

Printing Date : 22/10/2022 10:22:44

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Printed By : marwa mohamed youss

Patient Name: الأستاذة / عزه فاروق عبدالجنان على
Patient No.: 26905060102148
Path No.: 2542/22
Sex: Female
Age: 53 Y
Referring Doc.:

Request No.: 13009414
Request Date: 31/08/2022
Verification Date: 18/10/2022
Financial Category: اورام دار السلام - نفقة الدولة
Department: Out Patient
Registered by: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Ki67: Positive in 25% of tumor cells.

Prof. Dr. Lobna Omar.
Prof. Dr. Amal Hareedy.

Verified by :
Dr. Marwa mahmoud

Printing Date : 22/10/2022 10:22:44

Page 2 of 2

Printed By : marwa mohamed youss

البيانات الواردة في هذا التقرير هي لأغراض طبية فقط ولا يجوز استخدامها لأغراض أخرى.
The data in this report is for medical purposes only and should not be used for other purposes.

Women's Health Outreach Program

Name: ezza farook abdelhafez ali

Date: 30/8/2022

Bilateral Full field Digital Mammography With high resolution Ultrasonography.

Clinical summary: 55 y old female patient with history of right breast palpable lump

Technique

Mediolateral oblique and CC views

High resolution ultrasound

Detailed Findings

Right Breast

- Scattered fibroglandular densities (ACR b)
- UOQ rather circumscribed high density mass lesion with no overlying suspicious micro calcifications.
- by complementary ultrasound shows right retroareolar rather circumscribed hypoechoic mass lesion with internal echogenic foci of calcification, and mild vascularity by colour Doppler study measuring 30x15mm, touching the overlying skin, measuring 37x20mm seen at 9-10 o'clock position.
- Right MLN being 3cm apart from the mass.
- Normal skin/nipple/areolar complex
- one pathological axillary lymph nodes are seen at level II the largest measuring 29x12mm with focal cortical thickening reaching 6mm

left breast

- Scattered fibroglandular densities (ACR b)
- No suspicious masses, asymmetric densities, distortion or micro calcifications.
- No solid or cystic lesions
- Normal skin/nipple/areolar complex
- Non-specific axillary lymph nodes

Impression:

right breast: UOQ malignant looking solid mass with ipsilateral pathological lymph node

(BI-RADS 4b)

left breast: normal study (BI-RADS 1)

Best regards,

Dr. Anera abdel salam, FEBR, MSC.

JUSTAVE
HOUSSEY

Patient Name: Azza Farouk Abd Al Hafez Ali	Date : 02/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 56 Year(s)
Medical ID: 3702/2022	Pat. ID: 20004

RIGHT MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after left mastectomy.

- Breast density: Scattered densities of glandular tissue ...Breast density matches ACR B.
- No detected suspicious mass density or architectural distortion.
- No detected pathological micro-calcification.
- Normal skin and nipple.

Complementary breast ultrasound revealed:

- Homogenous (predominantly fatty) parenchymat echopattern of the right breast.
- No detected suspicious solid or cystic masses.
- No dilated ducts.
- Normal skin and nipple.
- No detected significant axillary lymph nodes .
- Clear surgical bed at the left side. No detected tumor residue or recurrence.

CONCLUSION:

- Right breast: Normal study BI-RADS 1 category.

Much Oblaed

DR.MONA ZAKY.MD

Professor of diagnostic radiology-Breast imaging unit -Mansoura University

Signature Dr. Mona Zaki

02/08/2025 12:40 PM

Patient Name: Azza Farouk Abd Al Hafez Ali	Date : 02/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 56 Year(s)
Medical ID: 3702/2022	Pat. ID: 20004

RIGHT MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: *Follow up after left mastectomy.*

- Breast density: Scattered densities of glandular tissue ...*Breast density matches* ACR 2.
- No detected suspicious mass density or architectural distortion.
- No detected pathological micro-calcification.
- Normal skin and nipple.

Complementary breast ultrasound revealed:

- Homogenous (predominantly fatty) parenchymal echopattern of the right breast.
- No detected suspicious solid or cystic masses.
- No dilated ducts.
- Normal skin and nipple.
- No detected significant axillary lymph nodes.
- Clear surgical bed at the left side. *No detected tumor residue or recurrence.*

CONCLUSION:

- *Right breast: Normal study E. RADS 1 category.*

Much Oblieed

DR. MONA ZAKI, MD

Professor of diagnostic radiology-Unit Imaging unit -Mansoura University

Signature: Dr. Mona Zaki

02/08/2025 12:40 PM



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المنشأة المصرية
للطب
MDT

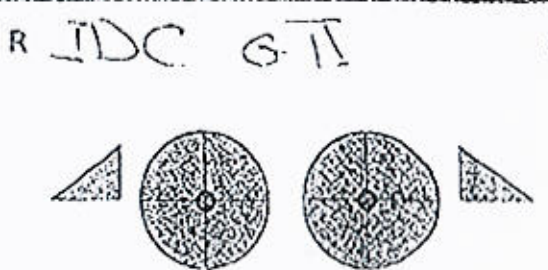
MDT FORM

٤ / ١٤ / ٢٠٢٢ G

Name	د. محمد عبد الله	MRN	C.C.C
Age	53 Y	ID	111 V a PAF L9
Menopausal status		Phone	079-0-7-1-0-0-0
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	Post MRM						
ER:	8/8	PR:	5/8	HER 2:	-ve	Ki67:	25%



Sonomammography:

Free margin

Mets Workup:

Free margin

- LN 1/5

TNM

YPT₂, PM₁

Recommendations:

Adjuvant therapy

upfront RT + TM

C.C.C / 11/12

MDT OUTCOME & PLAN

Adjuvant AC x 4 → Taxol x 12

+ Zoladex

Post RT - Cath insertion

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric